

Table 1: Cohort descriptive statistics across the five analytic cohorts (Table 1, C0 data anchor). Analytic samples are Medicare first-pair encounters (2015–2020); see build-funnel note below.

Characteristic	Opioid cohorts				Diabetes
	LBP	Cancer	Headache	Osteo [†]	T2D
<i>Analytic sample</i>					
Analytic <i>N</i> (encounters)	773,566	1,385,300	203,426	1,216,207 [†]	1,208,934
Unique patients	535,384	808,124	170,635	TODO	783,926
Unique physicians	90,996	118,900	67,260	TODO	132,620
<i>Patient characteristics</i>					
Age, mean (yr)	74.2	75.1	73.7	74.3	74.0
SD	6.67	6.98	TODO	6.94	TODO
Female, %	62.4	47.7	73.6	67.2	53.8
White, %	87.6	90.3	84.2	87.0	TODO
Black, %	5.7	4.3	6.4	TODO	TODO
LIS (low-income subsidy), %	17.1	12.2	22.0	17.0	27.1
Dual eligible, %	14.8	10.6	19.6	14.9	24.1
<i>Exposure: perceived Agreeableness</i>					
Agreeableness, mean	0.570	0.658	0.577	0.608	0.615
SD	0.39	0.38	0.40	0.38	0.39
<i>Prescribing outcome</i>					
Any opioid Rx, %	17.25	8.12	8.51	6.99	n/a
High-dose long-duration, %	13.77	3.94	5.97	TODO	n/a
Any diabetes Rx, %	n/a	n/a	n/a	n/a	56.00
Metformin Rx, %	n/a	n/a	n/a	n/a	26.91
<i>Selected comorbidities, % (6 most prevalent; full 22-flag control set in appendix)</i>					
Hypertension	70.3	TODO	TODO	67.4	TODO
RA / osteoarthritis	66.8	TODO	TODO	77.4	TODO
Hyperlipidemia	61.8	TODO	TODO	59.2	TODO
Ischemic heart disease	32.6	TODO	TODO	28.4	TODO
Diabetes	27.9	TODO	TODO	24.9	TODO
Anemia	27.8	TODO	TODO	27.8	TODO

Build funnel (data-pipeline cohorts; identical filter chain, encounters remaining): first-pair → anchor year 2016–2019 → age 65–95 → exclude cancer/hospice/ESRD → ≥180d coverage → 3-mo Rx coverage → survived 1 yr → MD/DO physician → 5–100 reviews. Endpoints: LBP 7,386,454 → 773,566; Cancer 15,658,490 → 1,385,300; Headache 3,216,764 → 203,426; T2D 14,539,965 → 1,208,934. The cancer-exclusion step is a no-op for the Cancer and T2D cohorts by design.

Note: One column per analytic cohort. Exposure is review-inferred physician Agreeableness (continuous ~0–1; *High* = score ≥ 0.75). The four opioid/diabetes data-pipeline cohorts share one identical first-pair filter chain and a single trait construction, supporting a combined Table 1; outcome rows differ by domain (opioid prescribing vs. diabetes-drug prescribing) and are not read across the opioid/T2D boundary. “n/a” denotes an outcome that does not apply to that domain. **TODO** marks an in-domain cell that is genuinely absent from *every* aggregated v0618 export (not missing data): the regression-stage logs for these cohorts carry only the variables their specifications use, so a descriptive that was never summarized cannot be recovered by parsing. Each such cell is **pending a server descriptives re-export**. The TODO cells are: Headache and T2D age SD; T2D White %; Osteo and T2D Black %; Osteo high-dose-long-duration %; Osteo unique-patient and unique-physician counts; and the six per-condition comorbidity prevalences for Cancer, Headache, and T2D (whose logs summarize only an aggregate `comorbid_count`, not the 22-flag per-condition set available for LBP and Osteo). [†] Osteoarthritis is **partial**: its figures come from the regression-stage logs (no data-pipeline build folder exists in this export), so it has no build funnel and its *N* is the fully-controlled regression *N* (loaded 1,216,207). Of the osteo cells, age (mean and SD), female, White % (mean `bene_race_white` = 0.870), LIS, dual, Agreeableness, and any-opioid-Rx % (`tab_is_opioid_rx` = 6.99) are read from the regression-stage `summarize/tab` output; unique-patient count, unique-physician count, and high-dose-long-duration % are not summarized at the regression stage and are marked **TODO** pending a server descriptives re-export. (The osteo OLS does report an `npi_id` cluster count of ~97,027, but that is the distinct-physician count of the controlled estimation sample, a different construct from the full-cohort unique-physician counts shown for the other columns, so it is not used here.) Source: v0618 `Report-From-CMS-Server` aggregated logs (probe P.0624-CD).